Restraint / Restrictive Practice Policy

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Policy Title: Restraint / Restrictive Practice

Effective Date: 2025-09-15

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Version: 1.0

Applies to: All staff, agency workers, contractors, volunteers, and residents at WM

1. Policy Statement

At WM, we are committed to safeguarding residents' rights, dignity, and autonomy. We recognise that restraint or restrictive practices should only ever be used as a last resort, when it is necessary, proportionate, and in the best interests of the resident.

We aim to:

- Minimise and, where possible, eliminate the use of restraint.
- Ensure any restrictive practice is legally and ethically justified.
- Protect residents from harm while upholding their human rights.
- Comply with the Mental Capacity Act 2005 (MCA), Deprivation of Liberty Safeguards (DoLS), and Human Rights Act 1998.

2. Purpose

This policy provides a framework to:

- Define restraint and restrictive practices.
- Guide staff on lawful and safe use of restraint.
- Ensure appropriate authorisation, recording, and monitoring.
- Promote alternatives to restrictive interventions.

3. Scope

This policy applies to all staff and covers:

- Physical Restraint – using physical force to control or restrict movement.
- Mechanical Restraint – using devices such as belts or bedrails.
- Chemical Restraint – using medication to control behaviour rather than treat a medical condition.
- Environmental Restraint – restricting movement by controlling access (e.g., locked doors).

4. Roles and Responsibilities

Registered Manager:

- Ensures compliance with legislation and CQC standards.
- Provides staff with training and guidance on restraint and alternatives.
- Reviews incidents, audits practice, and reports concerns.

Senior Staff / Nurses:

- Assess residents' capacity and best interests in line with the MCA.
- Ensure DoLS authorisation is obtained where required.
- Supervise and support staff in decision-making.

All Staff:

- Follow de-escalation and person-centred care approaches before considering restraint.
- Only use restraint if necessary, proportionate, and in line with training.
- Report and record all incidents immediately.

5. Principles for the Use of Restraint

- Last Resort – used only when less restrictive options have failed.
- Proportionality – the minimum necessary intervention for the shortest possible time.
- Best Interests – restraint must be in the resident's best interests under the MCA.
- Least Restrictive Option – always consider alternatives first.
- Dignity and Respect – residents must be treated with compassion and sensitivity.

6. Authorisation and Legal Framework

- Any use of restraint must comply with the Mental Capacity Act 2005 and the Human Rights Act 1998.
- If ongoing restrictions are required, a DoLS authorisation (or Liberty Protection Safeguards when enacted) must be sought.
- Chemical restraint must only be prescribed and reviewed by a qualified healthcare professional.
- Families and representatives must be informed unless this places the resident or others at greater risk.

7. Incident Response and Recording

Any use of restraint must be:

- Recorded in the resident's care plan and daily notes.
- Reported to the manager immediately.
- Reviewed to assess causes, effectiveness, and alternatives.

Serious incidents may need to be reported to:

- The CQC.

- The Local Safeguarding Adults Board.
- The Commissioning Authority.

8. Alternatives to Restrictive Practices

- De-escalation techniques (calm communication, reassurance).
- Environmental adaptations (safe spaces, noise reduction, personalisation).
- Distraction and engagement (activities, sensory interventions).
- Care planning to reduce triggers for distress or behaviour of concern.

9. Monitoring and Review

- All incidents of restraint will be monitored and analysed to identify patterns.
- Resident care plans reviewed regularly to reduce reliance on restrictions.
- Policy effectiveness reviewed through audits, incident analysis, and feedback.

10. Training

- All staff receive mandatory training in:
    - Mental Capacity Act and DoLS.
    - De-escalation and behaviour support.
    - Safe and lawful use of restraint (where appropriate).
- Refresher training provided annually or as needed.

11. References

- Mental Capacity Act 2005
- Deprivation of Liberty Safeguards (DoLS) – DoH Guidance
- Human Rights Act 1998
- CQC Fundamental Standards – Regulation 13 (Safeguarding service users from abuse and improper treatment)
- NICE Guideline NG10: Violence and aggression – short-term management

12. Related Policies

- Mental Capacity Act & DoLS Policy
- Safeguarding Adults Policy
- Risk Assessment Policy
- Incident Reporting Policy
- Mental Health & Wellbeing Policy
- Equality, Diversity & Inclusion Policy

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